

VETS MARKET INVESTIGATION

Summary of roundtable discussion held via MS Teams on 5 November 2024

Introduction

1. The following is a summary of points raised in a discussion with veterinary surgeons who work in practices owned by one of the six large corporate groups.
2. The roundtable discussion focused on the following issues:
 - Reflection on changes in the veterinary sector
 - Interactions with customers and contextualised care
 - Treatment options and pricing
 - The regulation of veterinary services, and how it might be improved

Reflections on changes in the veterinary sector

3. The majority of attendees said that there had been a considerable amount of change in the sector over the last five to ten years. Attendees discussed developments in: workforce shape and skills base, accessibility and use of different veterinary facilities (referral centres and hospitals), number and range of treatment options, equipment and diagnostic tools, client expectations and behaviours and public perception. We outline below and throughout the summary what the attendees said on these topics.
4. Some attendees noted that employees' rights to request part time or other working arrangements had changed the way practices were structured and staffed. They noted that employees were generally less willing to work unsociable hours than vets previously were. Some other attendees noted that these changes had contributed to the trend of out of hours care being provided by an external supplier outside the first opinion practice. One attendee said that flexible working and an increased focus on mental health had had a positive impact on wellbeing. Another attendee noted that the provision of better benefits like maternity leave had been led by large corporate groups.

5. Some attendees said that equipment and facilities had changed massively over the last ten years. One attendee said that the improvement in equipment had increased the quality and cost of pet care.
6. Some attendees discussed how the skills of new veterinary surgeons differed from those of previous generations. One attendee suggested that newly qualified vets may be less skilled than before as a result of increased referrals for procedures which previously may have been carried out by an FOP vet. Another attendee said that this had not been their experience, referring instead to attempts to upskill vets within the FOP setting, conscious of the lower cost of performing procedures within FOPs.

Interactions with customers and contextualised care

7. Most attendees noted that client expectations had increased. Some said that clients expected to be provided with multiple treatment options and a higher level of specialist care. One attendee said that they had experienced some clients looking for bespoke treatments for specific breeds at first opinion practices. Some other attendees said that, as a result of rising client expectations, there was a wider range of treatments on offer than were previously available. The attendees noted that some of these treatments were much more expensive than simpler treatments. One attendee noted that they had to change the way they communicate with some of their clients who enter with preconceptions; they believed that this was due to an increase in non-veterinary opinion leaders on social media. Another attendee said that social media had led to the credibility of the veterinary profession being questioned more.
8. One attendee told us that vets need to be trusted by their clients, and this is built through repeated interactions with the pet owner and animal over time. They noted that they would be concerned if an outcome of the CMA investigation was to encourage pet owners to shop around for different services. They said this could mean clients were less likely to discuss concerns with their vet and impact the ability of the vet to provide the best care in absence of the pets' medical history. Another attendee said that they would like pet owners to be aware that vets would be open to discussing costs and that they should not be embarrassed to do this or think they will be judged for asking.
9. There was a discussion on contextualised care which drew on an example of provision of out of hours care. Attendees outlined their practices' approaches to providing out of hours care when cost was a key factor for the pet owner. Some attendees said that, where cost was a factor, they offered clients the option for their pet to stay in the practice overnight but informed clients that

the pet would only be checked in the evening and then again in the morning. Other vet practices might offer for the pet to go home in the evening if they were well enough and then come back in the morning. One attendee said that there was a mismatch between the value provided by their services and what clients perceived the value to be. They noted that vets were not good at marketing the level of expertise and cost that went into their services, especially for out of hours care.

10. One attendee noted that, to provide contextualised care effectively, consultation times would need to increase. Another attendee noted the risk of informational overload if clients were to be provided with all the possible treatment options. They outlined research which suggested that, where a large volume of information and choices were put to people in an emotional situation, their ability to exercise informed consent was substantially compromised.

Treatment options and pricing

11. Some attendees said that they had freedom to decide prices for treatments within their first opinion practice. One attendee said that they did not have the ability to set prices and that this was set by the large corporate group, taking local factors into account.
12. There was a discussion on guidelines and protocols for treating common conditions. Some attendees noted the difference between protocols and guidance and were of the view that guidance would be more flexible and allow vets more flexibility in deciding on treatment options. Most attendees outlined that approaches to treating conditions would vary from one veterinarian to another. One attendee said that their team would have meetings to discuss approaches to provide consistency of advice for clients; the driver of this was due to clients' loss of trust where two vets provide different advice, even if both approaches are entirely valid.
13. One attendee noted that they had guidelines on how to approach conditions, but this was agreed locally, within the first opinion practice. Two attendees noted reluctance to work to protocols, as if the protocol directs the vet to do something contrary to the best interest of the client or professional obligations, they would not follow it. Two attendees said they had experienced less flexibility over treatment choices as an employee in an independent practice than in their current role: specifically, one attendee noted that when an FOP is owned by a practicing vet, they may have incentives to put in place protocols that have a positive financial impact on the business.

14. Some attendees outlined that the benefit of being part of a large corporate group was the access to a wider network of expertise and evidence to support decision making. One attendee said that their access to other practices' clinical outcomes was a key benefit for the wider profession and animal welfare.

Regulation

15. All attendees discussed ways in which regulation could better support the profession. Some attendees supported making the RCVS Practice Standards Scheme mandatory and ensuring that it was better communicated to, and understood by, consumers. One attendee said that the Practice Standards Scheme's current structure which was based on 'facility type' should be reformed. Another attendee said that this scheme, or an alternative, should aim to help consumers decipher between quality and service offerings.
16. Some attendees said that the RCVS was currently not able to regulate the profession as it only had the ability to regulate individuals and not practices. One attendee said that they had previously raised concerns about a practice in their local area and the RCVS were not able to take action.
17. One attendee mentioned an increase in friction with clients due to the recent changes in prescription rules for antiparasitic medications (ie flea and worming treatments) which mean that another consultation is needed (which vets will have to charge for) before a prescription can be provided, or the cascade provisions of the Veterinary Medicines Regulations 2013 (which covers the rules governing when, for example, a human version of a medicine may be prescribed by a vet for a household pet) means that pet owners face higher costs for medicines that are licenced for animals compared to the human version (which might have previously been used as part of the cascade).

Appendix: List of organisations represented at the discussion

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